SAMPLE ANSWER · DISTAL RADIUS FRACTURE · CASE 5

Sample 7-minute ISBAR: Lauren

Lauren Hughes · 37-year-old woman · Outpatient fracture clinic, metropolitan public hospital · about 1033 spoken words (≈ 7.4 min at 140 words per minute)

I Introduction

79 words

"Hi, I'm the occupational therapy student who has been seeing Lauren Hughes in the fracture clinic today. Lauren is a 37-year-old hairdresser with a left distal radius fracture, treated with closed reduction and a cast one week ago.

"I'd like to speak with you privately and promptly, because I've observed signs that make me concerned about Lauren's safety at home, and I need your help to give her a safe opportunity to speak with us alone before she leaves."

S Situation

157 words

"I was asked to review Lauren's cast comfort, everyday tasks and return to work. Her fracture position is maintained and her fingers are pink, warm and moving well.

"However, her partner, Craig, has answered most of my questions for her, including how she fell, and said he'd stay when I explained part of the appointment is one-on-one. When she took off her cardigan, I saw a row of four oval, fingertip-sized bruises on the inside of her right upper arm, some purple and some yellow-green. She covered them quickly and said she bruises easily.

"Craig said she 'missed the back steps carrying the washing', but the ED triage note records Lauren saying she 'tripped on the stairs inside'. Lauren is quiet, avoids eye contact and checks her phone every time it buzzes.

"Craig is now standing, saying they must leave in fifteen minutes for school pick-up, and I haven't had any chance to speak with Lauren alone."

B Background

153 words

"Lauren is right-handed and has worked as a hairdresser for twelve years, using her left hand to hold the comb and section hair. She fractured her left wrist one week ago, and it was treated with closed reduction and a below-elbow cast. She missed her first fracture clinic appointment four days ago, with no reason recorded.

"She lives with Craig and their two primary-school-aged children. When I asked about finances, she glanced at Craig and said, 'Money's Craig's department', and when I asked about showering and dressing she said 'Craig helps'. She has asked about styling hair in a cast, a cast cover and a rubbing cast edge. The only contact number on her file is Craig's.

"Gaps I'd want to clarify: whether a domestic and family violence screen was done in ED, whether the bruises were noted, any previous injury presentations, why she missed her appointment and what other supports she has."

A Assessment

262 words

"I'm concerned that Lauren may be experiencing domestic and family violence. I can't conclude that, and no single sign proves anything, but together these form a pattern that we have a responsibility to respond to: accounts of the injury that don't match; bruises of different ages in a pattern that could be consistent with being gripped; a partner who answers for her, insists on staying and is the only contact; a missed appointment; avoiding eye contact; checking her phone immediately; and 'money's Craig's department', which may suggest financial control.

"Craig is friendly and helpful, and that may be completely genuine, but it doesn't rule out a concern. Asking about violence in front of him, or confronting him, could increase the risk to Lauren after she leaves, so it's important I stay neutral with him.

"Occupationally, the fracture affects her work as a hairdresser, where her left hand holds and sections hair, and it increases her dependence on Craig for personal care, money and possibly transport, which can increase vulnerability. If she's unable to attend appointments freely, her fracture care could also be affected.

"There are children in the home, so their safety also needs consideration, and I'd need guidance about our obligations.

"Her questions about hairdressing, cast comfort and a cast cover are genuine and I'll address them, but they're not the priority. The priority is a safe chance to ask her privately. The WHO's LIVES approach — listen, inquire about needs, validate, enhance safety and support — fits what I'd hope to offer. As a student, I shouldn't manage this alone."

R Recommendation

382 words

"First, before they leave, I'd like your help to create a private opportunity. We could explain to both of them that it's routine for part of the review to be one-on-one, for example while we check her cast fit or adjust the cast edge, and ask Craig to wait outside. I wouldn't mention our concern to Craig or in front of him.

"If Lauren is alone, I'd like you or social work to ask her directly and calmly, following Queensland Health's domestic and family violence screening practice, whether she feels safe at home and whether anyone has hurt her. Whatever she says, we'd accept it without pressure, let her know help is available and, only if it's safe for her, give her contacts discreetly, such as DVConnect Womensline on 1800 811 811 or 1800RESPECT. If she discloses and is at immediate risk, we'd follow the service's DFV procedure, which may involve police. We'd also ask whether she has a safe number and time for us to contact her.

"Second, I'd ask social work to see her today if possible, or to follow up privately. Lauren decides what happens next, and we'd respect that.

"Third, I'd discuss with you and social work any child safety considerations and our obligations under Queensland Health policy.

"For documentation, I'd record objective observations and her exact words, including the bruises, the differing accounts and Craig's presence, and ask the medical team to examine and document the bruises. I'd make sure nothing sensitive appears in letters or summaries that go home.

"From the orthopaedic team, I'd ask them to check the cast edge rubbing near her thumb and confirm her restrictions for work.

> "For OT assessment, my principal short-term assessment would be observed performance of essential daily tasks within her cast precautions, such as dressing, showering set-up and preparing a simple meal, which is best done one-on-one and creates another private opportunity. Longer term, I'd use the COPM, which gives Lauren her own voice about her priorities, such as returning to hairdressing and caring for her children.
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> "Intervention would include cast care, a cast cover if appropriate, and graded return-to-work planning once restrictions are confirmed. I'd book follow-up appointments directly with her where safe. Today, the priority is a safe, private opportunity for Lauren to speak with us."

Coaching notes *not spoken*

The immediate priority

A cluster of signs suggests possible domestic and family violence. The priority is to create a safe, private opportunity to ask Lauren alone, without confronting Craig, and to involve the supervisor and social work before she leaves.

Distractors in this case

- **Craig's friendly, helpful manner** — does not exclude risk; stay neutral and never raise concerns in front of him.
- **Satisfactory X-ray and cast** — the fracture is stable, but that is not the main issue today.
- **Hairdressing, cast comfort and cast-cover requests** — genuine and useful for building rapport, but secondary.
- **School pick-up deadline** — real pressure; use a routine one-on-one cast check to create time.

Missing information to clarify

- Whether DFV screening was done in ED and whether bruises were documented.
- Previous injury presentations and reasons for the missed appointment.
- Lauren's own safe contact details and supports.
- Child safety considerations.

Useful references

- World Health Organization (2014). *Health Care for Women Subjected to Intimate Partner Violence or Sexual Violence: A Clinical Handbook* (LIVES).
- DVConnect Womensline (1800 811 811) and 1800RESPECT (1800 737 732).

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